

UUID: 9F0E20CB-BCF4-4C9D-AB94-792EE20D5033
TCGA-L6-A4EP-01A-PR

Redacted

Gender: [REDACTED]

Birth Date: [REDACTED]

Admit Protocol: [REDACTED]



Final Results

Surgical Pathology

Final

CASE NUMBER: [REDACTED]

DIAGNOSIS:

1. Lymph node, delphian, biopsy: Lymph nodes, no tumor seen.
2. Thyroid, right thyroid nodule, frozen section (1FS1): Papillary thyroid carcinoma.
3. Thyroid, right lobe and isthmus, excision: Papillary thyroid carcinoma (3.0 cm in greatest dimension). Please see CAP check list below.

Procedure

Right Thyroid lobectomy

Received:

Fresh

Specimen Integrity

Intact

Specimen Size

Right lobe: 5.4 x 3 x 2.5 cm

Specimen Weight

Specify: 14.9 g

Tumor Focality

Cannot evaluate

Right lobe/isthmus only

Dominant Tumor

Tumor Laterality

Right lobe

Tumor Size

Greatest dimension: 3 cm

Additional dimensions: 2.4 x 2.4 cm

Histologic Type

Papillary carcinoma

Variant, specify:

Classical (usual)

Architecture:

Classical (papillary)

Cytomorphology:

Classical

Margins

Margins uninvolved by carcinoma (tumor closely approaches but is not present at inked margin)

ICD - 0 - 3

Carcinoma, papillary, thyroid 8260/3

Site: thyroid, nos C73.9

hw
9/28/12

Gender: [REDACTED]

Birth Date: [REDACTED]

Admit Protocol: [REDACTED]

Admit Protocol: [REDACTED]

Admit Protocol: [REDACTED]

Age: [REDACTED]

Final Results

Tumor Capsule

Totally encapsulated

Tumor Capsular Invasion

Present: Multiple foci

Lymph-Vascular Invasion

Not identified

Extrathyroidal Extension

Not identified

Pathologic Staging

Primary Tumor

pT2: Tumor more than 2 cm, but not more than 4 cm, limited to thyroid

Regional Lymph Nodes

pN0: No regional lymph node metastasis

Specify: Number examined: 3

Number involved: 0

Distant Metastasis

Not applicable

Additional Pathologic Findings

One Parathyroid gland: Slightly hypercellular

Immunoperoxidase and in-situ hybridization tests performed here and used for diagnosis were developed and their performance characteristics determined by the Laboratory of Pathology, . . . They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has also determined that such clearance or approval is not necessary. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing.

CLINICAL INFORMATION: Allocate Order to Protocol: [REDACTED] Brief

Clinical History: right

thyroid nodule Specimen Taken For Protocol: 01 - Yes

PROCEDURE: Pre-Operative Diagnosis: as above Post-Operative

Diagnosis: as above

Operative Findings: as above

SPECIMENS SUBMITTED: 1. SERIAL 5+ 5FS, Delphian lymph node 1FS)

2. ROUTINE + 4FS, Right thyroid nodule (2FS)

3. THYROID, RIGHT LOBE, and Isthmus

INTRAOPERATIVE CONSULTATION: Received fresh from the Operating Room with the patient's name, medical record number, and labeled as:

Confidential Patient Information. Unauthorized disclosure is prohibited by the Federal Privacy Act of 1974.

Requested By: [REDACTED] (Other)

Do not file in Medical Record

[REDACTED]		Admit Protocol: [REDACTED]
Gender: [REDACTED]	Birth Date: [REDACTED]	Age: [REDACTED] y
Admit Protocol: [REDACTED]	Admit Protocol: [REDACTED]	

Final Results

1. "Delphian lymph node" is a red soft tissue fragment measuring 0.6 x 0.4 x 0.3 cm. It is frozen in total as 1FS.

Preliminary diagnosis: Lymph node, no definitive tumor seen.

2. "Right thyroid nodule, rule out cancer", consists of a yellow piece of soft tissue fragment measuring 2.1 x 1.2 x 0.5 cm. It is totally frozen as 2FS (4 slides).

Preliminary diagnosis: Follicular papillary lesion.

This is performed by

GROSS DESCRIPTION: Three specimens are received in formalin, labeled with the patient's name, medical record number, and further specified as follows:

1. "Delphian lymph node". The specimen is received in Pathology and matches the intraoperative consultation note. The specimen is transferred from a green cassette labeled with the patient's name and 1FS to an orange cassette labeled [REDACTED].
2. "Right thyroid nodule 2FS". The specimen is received in Pathology and matches the above description. The specimen is transferred from a green cassette labeled with the patient's name and date to an orange cassette labeled [REDACTED].
3. "Right thyroid lobe and isthmus, stitch marks superior pole" is a partial thyroidectomy specimen consisting of a right thyroid lobe and portion of isthmus, overall 14.9 grams. The right thyroid lobe is 5.4 x 3 x 2.5 cm. The exterior surface is inked black. The posterior surface is over-inked blue. The right thyroid lobe has a nodule, 3 x 2.4 x 2.4 cm. The nodule has a friable tan-pink glistening cut surface. Gross photos are taken. The procurement is performed by Dr. [REDACTED] and [REDACTED] on [REDACTED] at [REDACTED]. Approximately 50% of the right nodule is procured for Dr. [REDACTED] and Dr. [REDACTED]. Approximately 0.8 x 0.5 x 0.5 cm of normal-appearing thyroid from the isthmus is procured for research with half for Dr. [REDACTED] and half for [REDACTED]. The remainder is sent to Pathology for permanent processing. The specimen is received in Pathology and matches the above description. The isthmus is not readily identifiable. The specimen is sectioned from the superior aspect marked by the double stitch to the inferior aspect. Every other segment of the thyroid going from superior to inferior is submitted in orange cassettes labeled [REDACTED]. The specimens will be processed as a rush. Intervening segments going from superior to inferior are submitted in white cassettes labeled #3K-3P. These will be held overnight in formalin before further processing.

No consultants

Confidential Patient Information. Unauthorized disclosure is prohibited by the Federal Privacy Act of 1974.

Requested By: [REDACTED] (Other)

Do not file in Medical Record

Clinical Center

[Redacted]		Admit Protocol: [Redacted]
Gender: [Redacted]	Birth Date: [Redacted]	Age: [Redacted]
Admit Protocol: [Redacted]	Admit Protocol: [Redacted]	
Final Results		

<Resident Signature>

<Sign Out Dr. Signature>

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed	

9/9/12

Confidential Patient Information. Unauthorized disclosure is prohibited by the Federal Privacy Act of 1974.

Requested By: [Redacted] (Other)

Do not file in Medical Record